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<u>HHS Health Sector AI RFI</u>

Thank you for the opportunity to comment on HHS's Request for Information on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care. These comments focus on concrete, legally grounded steps HHS can take—through regulation, reimbursement, and research and development—to reduce barriers, promote trustworthy adoption, and align incentives, consistent with HIPAA and the ONC information blocking framework.

I. Executive summary

HHS should advance a forward-leaning, industry-supportive approach to clinical AI by: (1) removing practical and economic barriers to accessing high-value clinical data for privacy-preserving model development and evaluation, with targeted incentives for health record holders and electronic health record ("EHR") developers; (2) facilitating creation of "data twins" programs and systems to generate robust, de-identified real-world data assets while protecting patient privacy and ensuring patient autonomy via opt-out rights and permissioned compensation pathways; (3) promulgating best-practice guidance for meaningful human review in clinical AI, emphasizing privacy and security safeguards post-deployment; (4) addressing workflow integration and EHR friction as fundamental barriers to sustainable clinical AI adoption; and (5) clarifying HIPAA terms—including business associate "management and administration"—to define permissible AI training and operations under business associate agreements (BAAs), while preserving patients' trust and rights. These recommendations align with the RFI's goals to use regulation, reimbursement, and R&D to accelerate adoption, lower burden and cost, and improve outcomes.

II. Barriers and Recommendations in Alignment with HHS priorities

HHS's RFI seeks input to establish a clear, risk-based regulatory posture and to use reimbursement and R&D to enable safe, effective clinical AI at scale. This complements the Department's "OneHHS" AI Strategy and calls for concrete, experience-based feedback to ensure rapid adoption while protecting privacy and public trust. The RFI specifically invites recommendations on barriers to innovation, regulatory changes and definitions, evaluation methods for non-device AI, and ways to support accreditation/certification and interoperability—all central to the proposals below.

Barrier 1: Limitations on ability to access accurate and complete clinical data across systems restrict the development of robust, real-world datasets within the health care ecosystem

From a clinical perspective, data access barriers extend beyond legal and technical interoperability. Even when fast healthcare interoperability ("FHIR") application programming interfaces ("APIs") are used, data quality is often inconsistent (for example, often missing problem lists, medications, and clinical notes) and APIs do not reliably expose the data types needed for meaningful AI applications, including clinical notes, imaging narratives, prior authorization data, and longitudinal context. Cost, contracting complexities, and "security reviews" can function as de facto blocking mechanisms, and tools that require extra clicks, toggling, copy/paste operations, or dual documentation will not survive in production environments. "Pilot success" frequently does not translate to sustainable deployment without deep EHR integration and reliable uptime.

Recommendation 1: Leverage ONC's interoperability and information blocking frameworks to remove barriers to access clinical data across EHR systems while continuing to protect patient privacy

HHS should leverage ONC's interoperability and information blocking frameworks to reduce friction and enable responsible access, exchange, and use of electronic health information (EHI) for clinical AI development and evaluation (i) among covered entities and business associates subject to HIPAA (including minimum necessary) requirements; and (ii) outside the realm of HIPAA protection when consistent with HIPAA de-identification standards. Specifically:

- Establish targeted incentives and reimbursement supports. HHS should explore payment policy options and grant mechanisms that reward providers, health information networks, and EHR developers for enabling standardized, automated access to de-identified and limited datasets for model development and post-deployment monitoring, consistent with information blocking rules and the Manner Exception. Aligning incentives with TEFCA participation and USCDI expansion would widen market opportunities and accelerate AI development, as the RFI explicitly seeks.

- Clarify that practices impeding automated, standards-based data access may constitute information blocking absent an applicable exception. ONC has already highlighted that practices interfering with automation technology's ability to access, exchange, or use EHI are of concern; HHS should reinforce this through FAQs and guidance to support AI model lifecycle needs while ensuring privacy and security.

- Encourage standardized de-identified data export APIs. HHS should consider ONC certification criteria enhancements to ensure certified health IT can generate and transmit de-identified datasets using consistent standards (e.g., FHIR bulk export profiles) to support research and model evaluation, subject to governance controls and HIPAA de-identification.

- Support real-world evaluation through data access. FDA's ongoing inquiry into measuring and evaluating AI-enabled tools in the real world underscores the need for reliable post-deployment data access to assess model drift, bias, and safety—principles that also apply to non-device clinical AI.

- Strengthen interoperability expectations for specific high-value data types and benchmark data completeness, not just API presence.

- Create ONC/CMS incentives that favor measurable workflow impact such as time saved and reduction in inbox burden, rather than generic "AI available" metrics.

These steps would operationalize the RFI's emphasis on a predictable, proportionate regulatory posture and on aligning federal levers while expediting the creation of robust, real-world data sets to enhance the reliability of AI tools.

Barrier 2: Use of personal identifiers without patient or consumer knowledge or consent erodes trust and undermines individual autonomy

Clinicians increasingly report that patients ask questions such as "Is an AI tool involved?" and "Did you use my data to train it?" Concerns are particularly acute for sensitive topics including behavioral health, reproductive care, and HIV/SUD data. **Recommendation 2: Adopt patient consent standards and facilitate privacy-protective "data twins" with patient opt-out and permissioned compensation**

HHS should provide clearer public-facing guidance and clinician-facing scripts for consent and notice in specific clinical contexts.

To scale high-quality, real-world datasets while protecting privacy, HHS should encourage the creation of "data twins"—de-identified datasets that mirror health attributes of real persons—under robust governance:

- Establish a clear policy framework for data twins. HHS should issue guidance clarifying how HIPAA de-identification and health care operations concepts apply to creating de-identified data twins for quality improvement, safety monitoring, and AI development, with guardrails for re-identification risk management and vendor accountability.

- Require transparent notices and a simple opt-out. HIPAA does not protect de-identified data, but HHS should encourage transparent patient notices and a standardized opt-out for the creation or use of a data twin, balancing trust and innovation consistent with the RFI's focus on public confidence.

- Permit compensation pathways with safeguards. HHS should clarify that programs offering remuneration to individuals for permitting use of their data for twin creation (where applicable under state law and program design) can proceed with appropriate disclosures, anti-exploitation protections, and without conditioning treatment or benefits—promoting inclusion and representativeness in datasets.

- Promote interoperability for data twins. ONC should align USCDI/USCDI+ and TEFCA policies to support standardized, de-identified data assembly for twins, enabling benchmarking and external validation while upholding information blocking's pro-interoperability aims.

Together, these steps would advance trustworthy data generation for AI while respecting patient autonomy and privacy expectations articulated across HHS's AI posture.

Barrier 3: Lack of industry-wide standards for human review, oversight, evaluation, and description of clinical AI tools

The industry lacks standards for clinician involvement in review, oversight, evaluation, and description of clinical AI tools throughout the tools' lifecycles. Clinicians face significant trust barriers when AI model outputs are not explainable to patients in terms of clinical rationale and evidence anchors. Additionally, providers will not adopt tools that change silently through new versions with new behaviors. **Recommendation 3: Issue best-practice guidance for human review, post-deployment oversight, evaluation, and decription of clinical AI tools**

HHS should promulgate cross-industry guidelines for non-device clinical AI on meaningful human involvement, privacy/security controls, and continuous model oversight:

- "Three-touch" human-in-the-loop for clinical AI decision-support tools. Require expert review prior to deployment (to include review of data provenance and labeling, model outputs, etc.); at early deployment (pilot validation); and post-deployment (ongoing monitoring and governance).

- Human-in-the-loop for consequential decisions. Building on stakeholder input and payer-side constraints that already require clinician involvement, HHS should require AI-influenced determinations affecting access to services or clinical care receive timely, qualified human review with appropriate documentation, reducing inappropriate denials and supporting due process.

- Pre- and post-deployment monitoring. HHS should encourage routine monitoring for model drift, bias, and performance across populations and settings, leveraging both automated and human-expert review approaches tailored to the workflow, consistent with FDA's focus on real-world performance. HHS should publish a minimum evaluation and monitoring checklist for clinical AI, covering both pre-deployment and post-deployment requirements.

- Interoperability and measurement readiness. HHS should tie R&D support to the use of standardized data elements (USCDI) and exchange frameworks (TEFCA) to enable replicable evaluations and equitable performance assessments.

- "Change transparency." HHS should issue requirements for clinical AI tools to monitor and provide notice of changes to the tools, including release notes, notification protocols, and re-validation requirements when models are updated.

This guidance would complement the RFI's aims to foster public trust and reduce uncertainty that impedes innovation, particularly for non-device AI workflows in clinical care.

Barrier 4: Lack of regulatory guidance regarding use of AI in compliance with HIPAA

Clinicians need regulatory clarity related to various legal issues, including for example the standard of care when AI is used; what constitutes reasonable reliance versus independent verification; malpractice risk when AI recommendations are wrong, missing, or biased; and documentation expectations (e.g., inclusion in HIPAA notice of privacy practices; patient consent). In addition, HIPAA lacks clarity as to the ability of a business associate to use patient data for purposes of training its own (or a subcontractor's) clinical AI tool.

Recommendation 4: Develop model guidance and safe-harbor concepts for documented, risk-tiered AI use; clarify HIPAA terms to define permissible AI uses by covered entities and business associates; issue HIPAA security guidelines to simplify procurement.

To remove ambiguity that slows responsible adoption, HHS should issue clarifications and model BAA language addressing:

- AI training and "management and administration." HHS should clarify when a business associate's training, tuning, or maintenance of AI models using PHI qualifies as "the proper management and administration" of the business associate, and when separate authorization or de-identification is required.

- Purpose limitation and data minimization. HHS should reinforce that business associate-conducted AI activities must adhere to purpose limitations, minimum necessary, and prohibitions on impermissible secondary uses such as marketing.

- Transparency and patient notices. HHS should include language regarding use of PHI in connection with AI in the model Notice of Privacy Practices and describe permissible uses by covered entities, including de-identified uses, consistent with building trust while enabling efficient operations under existing HIPAA pathways.

- Security friction. HHS should issue security guidelines and/or create standardized procurement and security templates specific to clinical AI to alleviate the burden on smaller organizations that lack resources to navigate complex vendor onboarding processes.

- Harmonization with information blocking. HHS should confirm that HIPAA-compliant privacy/security controls and BAA requirements can co-exist with the obligation not to engage in information blocking, and that reasonable, standardized safeguards do not constitute improper interference with EHI access, exchange, or use.

These clarifications would help standardize contracting and compliance expectations, reducing uncertainty and transaction costs across the health care ecosystem.

III. General Comments re: Reimbursement and Research and Development (R&D)

- Reimbursement pilots for validated AI. CMS should test payment models that reimburse for the acquisition and use of AI that measurably reduces burden or improves outcomes, with requirements for post-deployment monitoring and equitable performance, as contemplated by the RFI's reimbursement focus. CMS should align payment so that high-value AI-enabled workflows are financially viable—whether through time-based codes, value-based care incentives, or other payment mechanism that encourages adoption of high-value AI tools.

- R&D to build shared evaluation infrastructure. HHS should fund reference datasets, benchmarking tools, and secure enclaves to support external validation and fairness testing for non-device AI, aligning with FDA's real-world performance focus and ONC interoperability priorities.
- Support for accreditation and certification. HHS should catalyze private-sector accreditation and certification programs for clinical AI governance and post-market oversight, promoting consistency without duplicative regulation.

These investments directly address the RFI's call to align regulation, reimbursement, and R&D to accelerate adoption and lower costs while preserving safety and privacy.

IV. Conclusion

HHS can accelerate trustworthy clinical AI adoption by removing data access barriers through interoperability and targeted incentives; addressing workflow integration as a fundamental requirement for sustainable deployment; establishing a patient-respectful framework for de-identified data twins; issuing best-practice guidance for human review and privacy-preserving post-deployment oversight; establishing minimum evaluation standards for non-device AI; addressing reimbursement misalignment that burdens clinicians; streamlining procurement and security processes; and clarifying HIPAA terms to define permissible AI uses by covered entities and business associates. These steps are squarely within the RFI's vision to deploy AI in ways that enhance productivity, reduce burden, lower costs, and improve outcomes while maintaining public trust and confidentiality.

Submitted in response to: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care; comments due February 23, 2026, via Regulations.gov or mail/courier as specified.

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